

MEDSPA STANDARDS

PROTOCOL KIT

Weight Loss Protocols

Ten protocols spanning the full GLP-1 patient journey — from the consultation that screens someone out, through titration and side effect management, to the day they stop.

10 SOPS · VERSION 1.0 · EFFECTIVE JANUARY 2026

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Before you put these into service

1. Have your medical director set the practice-specific figures the protocols call for: BMI thresholds, protein targets, lab panels and intervals, and the pregnancy discontinuation interval.
2. Have a healthcare attorney review WL-006 and confirm the basis on which compounded product is used, with a dated review date.
3. Complete the pharmacy vetting file in WL-006 before the next order — not retrospectively.

4. Adopt one disordered eating screening instrument (WL-003) and identify a named referral clinician.
5. Add the regain counseling in WL-010 to your intake conversation this week; it belongs at the start, not the exit.
6. Print the quick reference cards, laminate them, and post them where consultations happen.

WEIGHT LOSS

Semaglutide Protocol

Candidacy, titration, administration, monitoring and the documentation that makes this a medical program

DOCUMENT WL-001	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol governs semaglutide therapy for weight management from initial candidacy through maintenance. It applies to FDA-approved branded product and, where the practice uses it, to compounded semaglutide — with the additional requirements in WL-006 applying in full to the latter.

ABSOLUTE CONTRAINDICATIONS

Personal or family history of medullary thyroid carcinoma, or Multiple Endocrine Neoplasia syndrome type 2. Known hypersensitivity to semaglutide or any excipient. Ask the question explicitly, record the question and the answer, and do not proceed on a shrug.

THIS IS A PRESCRIBING RELATIONSHIP

Semaglutide is a prescription medication. A questionnaire and a monthly vial is not a medical program and is precisely the model regulators are moving against. Every element below — evaluation, documented rationale, monitoring, follow-up — is what distinguishes the two.

2 Candidacy & Baseline Evaluation

Full candidacy criteria are in WL-003. Before a first prescription is written, all of the following must be complete and in the chart.

- ☐ Height, weight and BMI measured in the practice — not self-reported.
- ☐ Blood pressure and resting heart rate recorded.
- ☐ Complete medication list including insulin, sulfonylureas, oral contraceptives and thyroid medication.
- ☐ Personal and family history screen for medullary thyroid carcinoma and MEN2, documented.
- ☐ History of pancreatitis, gallbladder disease, gastroparesis or bariatric surgery.
- ☐ Diabetic retinopathy status in patients with diabetes.
- ☐ Pregnancy status, contraception, and pregnancy intent discussed and recorded.
- ☐ Disordered eating screen completed per WL-003.
- ☐ Baseline laboratory panel drawn and reviewed per WL-009.
- ☐ Informed consent covering off-label or compounded status where applicable.

PREGNANCY PLANNING

Semaglutide is not used in pregnancy and should be discontinued well in advance of a planned conception because of its long half-life — your medical director must specify the discontinuation interval used by the practice. Note also that weight loss and delayed gastric emptying can affect oral contraceptive reliability; counsel patients accordingly and document it.

3 Titration Schedule

The escalation schedule below follows the labeled approach for weight management. **Verify against current manufacturer labeling before adoption**, and have your medical director confirm the schedule used for any compounded preparation, which may differ in concentration and presentation.

STANDARD WEIGHT-MANAGEMENT ESCALATION

Phase	Weekly dose	Minimum duration	Notes
Initiation	0.25 mg	4 weeks	Tolerance-building dose. Not intended to produce meaningful weight loss.
Escalation 1	0.5 mg	4 weeks	Assess tolerance before advancing.
Escalation 2	1.0 mg	4 weeks	Many patients respond adequately here.
Escalation 3	1.7 mg	4 weeks	Advance only if response is inadequate and tolerance is good.
Maintenance	2.4 mg	Ongoing	Maximum labeled weight-management dose. Do not exceed.

FOUR WEEKS IS A FLOOR, NOT A TARGET

The minimum interval exists to build tolerance and reduce GI adverse effects. Escalating faster because a patient is impatient or a plateau feels slow is the most common avoidable error in this category. If a patient requests faster escalation, decline, explain, and document both. See WL-004.

4 Administration & Storage

- 1

Confirm the product and concentration against the prescription before drawing or dispensing. Concentration varies between branded pens and compounded vials, and this is where the serious dosing errors happen.
- 2

Verify the dose in milligrams, then convert to volume. Never accept a dose expressed only in syringe 'units' — insulin-syringe unit markings are a volume scale, not a semaglutide dose, and mixing the two has caused overdoses.
- 3

Select the injection site — abdomen, thigh or upper arm — and rotate sites between doses. Record the site used.
- 4

Inject subcutaneously using appropriate technique. Same day each week; the time of day does not matter.
- 5

Record product, manufacturer or compounding pharmacy, lot number, beyond-use or expiry date, dose, site and administering clinician.
- 6

Train the patient if self-administering: technique demonstrated, teach-back obtained, written instructions supplied, sharps disposal explained. Record the training.

Item	Standard
Storage	Refrigerated per labeling. Do not freeze; discard if frozen. Protect from light.
In-use dating	Per manufacturer labeling for branded product; per the compounding pharmacy's beyond-use date for compounded preparations. Whichever is shorter governs.
Cold chain	Temperature log maintained for the practice refrigerator, checked and recorded daily.
Excursion	Any out-of-range event assessed and documented; product quarantined pending a disposition decision by the medical director.

Missed doses

- A dose missed by a short interval may generally be taken as soon as remembered, then the usual weekly schedule resumed — confirm the specific window against current labeling.
- If more than two consecutive weekly doses are missed, do not simply resume the previous dose. Re-initiation at a lower dose and re-titration is usually required because tolerance is lost.
- Record every missed dose, the gap length, and the restart decision with its reasoning.

5 Ongoing Assessment

At every visit	Assess and record
Response	Weight, percentage change from baseline, and progress against the goal agreed at intake.
Tolerance	GI symptoms graded per WL-005. Ask specifically rather than waiting for the patient to volunteer.
Vitals	Blood pressure and resting heart rate. A sustained rise in heart rate is a known effect and warrants attention.
Nutrition	Protein intake and adequacy of oral intake. Very low intake is a clinical finding, not a success.
Red flags	Severe abdominal pain radiating to the back, persistent vomiting, signs of gallbladder disease, vision change in diabetic patients.
Adherence	Doses taken, missed doses, self-administration technique if applicable.

STOP AND REFER TODAY

Severe persistent abdominal pain, particularly radiating to the back and with vomiting, requires same-day medical evaluation to exclude pancreatitis. Hold the drug. Do not reschedule the patient for next week.

ANESTHESIA AND SEDATION

Delayed gastric emptying raises aspiration risk under sedation or general anesthesia. Instruct every patient to inform any surgeon, anesthesiologist, dentist or endoscopist that they are on a GLP-1 agent, and record that you gave that instruction. This applies to procedures in your own practice too.

6 Documentation Requirements

- Candidacy criteria met, with the specific findings that support the decision.
- Contraindication screen: the questions asked and the answers given.
- Baseline labs, vitals and measurements with dates.
- Informed consent, including off-label or compounded disclosure where applicable.
- Every dose: date, product, source, lot, beyond-use date, amount, site, administrator.
- Every escalation decision with the criteria applied — and every decision not to escalate.
- Adverse effects reported, grading applied, and action taken.
- Patient education delivered, including the anesthesia warning and sharps disposal.
- Follow-up interval set, and action taken if the patient does not attend.

7 Quick Reference

QUICK REFERENCE CARD

Semaglutide — every visit

1. **Weight, BP, resting heart rate.** Measured, not reported.
2. **Ask about GI symptoms directly.** Grade per WL-005.
3. **Confirm protein and fluid intake.** Very low intake is a finding.
4. **Escalate only at 4+ weeks** and only against the criteria in WL-004.
5. **Record product, source, lot and BUD** for every dose.
6. **Severe abdominal pain radiating to back → hold and refer same day.**
7. **Remind: tell any anesthetist or surgeon** you are on a GLP-1.

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
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WEIGHT LOSS

Tirzepatide Protocol

Dual GIP/GLP-1 therapy: titration, cross-titration from semaglutide, and monitoring

DOCUMENT WL-002	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Tirzepatide acts on both GIP and GLP-1 receptors and has its own titration ladder, tolerance profile and maximum dose. It is not interchangeable with semaglutide on a milligram basis and must never be substituted or converted arithmetically.

ABSOLUTE CONTRAINDICATIONS

Personal or family history of medullary thyroid carcinoma, or MEN2. Known hypersensitivity. Identical screening to semaglutide — ask explicitly and document the answer.

Candidacy, baseline evaluation, storage, cold chain, administration technique and documentation requirements follow WL-001 unless stated otherwise below. This protocol covers what differs.

2 Titration Schedule

Verify against current manufacturer labeling before adoption. Escalation is slower and longer than semaglutide, with more steps.

STANDARD WEIGHT-MANAGEMENT ESCALATION

Phase	Weekly dose	Minimum duration	Notes
Initiation	2.5 mg	4 weeks	Tolerance-building dose only.
Escalation 1	5 mg	4 weeks	First therapeutic dose. Many patients do well here.
Escalation 2	7.5 mg	4 weeks	Advance only for inadequate response with good tolerance.
Escalation 3	10 mg	4 weeks	Reassess whether further escalation is warranted.
Escalation 4	12.5 mg	4 weeks	Second-line maintenance option.
Maximum	15 mg	Ongoing	Maximum labeled dose. Do not exceed.

MAINTENANCE IS NOT ALWAYS THE MAXIMUM

A patient meeting their goal comfortably at 7.5 mg does not need 15 mg. The maximum dose is a ceiling, not a destination. Record the reasoning for the maintenance dose chosen.

3 Cross-Titration from Semaglutide

NO DOSE CONVERSION EXISTS

There is no validated milligram-to-milligram equivalence between semaglutide and tirzepatide. Converting 2.4 mg semaglutide into a 'comparable' tirzepatide dose is not supported and risks significant adverse effects. Switch by starting the tirzepatide ladder at an appropriate initiation dose.

- 1 **Document the reason for switching** — inadequate response, intolerance, supply, or patient preference. This is a clinical decision, not an upgrade.
- 2 **Stop semaglutide.** Take the accumulated effect of its long half-life into account when timing the first tirzepatide dose; your medical director sets the practice interval.
- 3 **Start tirzepatide at an initiation dose** appropriate to the patient's tolerance history, as determined by the prescriber. Do not start at a high dose because the patient 'tolerated semaglutide fine.'
- 4 **Re-titrate on the standard schedule** with the same minimum intervals.
- 5 **Monitor closely through the first two escalations** — overlapping residual effect plus a new agent is the highest-risk window for GI adverse effects.
- 6 **Record the full rationale**, the interval used, and the starting dose chosen.

4 Response & Plateau

Set expectations at intake, not at the plateau. Weight loss is not linear, it slows over time, and a plateau is a physiological adaptation rather than a treatment failure.

Situation	Assessment	Action
Steady loss, good tolerance	Therapy working as intended.	Hold current dose. Do not escalate purely to accelerate.
Slowing rate after several months	Expected adaptation.	Reinforce nutrition and resistance training per WL-007. Reassess goal. Escalate only if criteria in WL-004 are met.
Genuine plateau at submaximal dose with good tolerance	Inadequate response.	Consider escalation per WL-004 after confirming adherence and reviewing intake.
Plateau at maximum dose	Maximum benefit from this agent reached.	Discuss maintenance, alternative agents, or referral. Do not exceed the labeled maximum.
Weight loss with poor oral intake or lean mass loss	Adverse, not successful.	Address intake and protein per WL-007. Consider dose reduction.

5 Monitoring & Red Flags

- Assessment at every visit as per WL-001, including resting heart rate.

- Laboratory monitoring per WL-009.
- GI symptom grading per WL-005 at every contact.
- Hypoglycemia risk is meaningful only in combination with insulin or a sulfonylurea — identify those patients and coordinate with their prescriber before initiation.
- Gallbladder symptoms, particularly during rapid weight loss.
- Injection site reactions, which are more commonly reported with this agent.

SAME STOP CRITERIA AS WL-001

Severe abdominal pain radiating to the back with vomiting, persistent vomiting with dehydration, signs of bowel obstruction, or a suspected allergic reaction — hold the drug and arrange same-day medical evaluation.

ANESTHESIA WARNING APPLIES EQUALLY

Delayed gastric emptying and aspiration risk under sedation apply to tirzepatide as much as to semaglutide. Give the instruction, and record that you gave it.

6 Quick Reference

QUICK REFERENCE CARD

Tirzepatide essentials

1. **Ladder:** 2.5 → 5 → 7.5 → 10 → 12.5 → 15 mg. Four weeks minimum per step.
2. **Maximum is a ceiling, not a target.** Document the maintenance dose chosen.
3. **No conversion from semaglutide.** Restart the ladder.
4. **Highest-risk window** is the first two steps after a switch.
5. **Insulin or sulfonylurea on board?** Coordinate before initiating.
6. **Same stop criteria** as WL-001. Same anesthesia warning.
7. **Confirm mg, then convert to volume.** Never dose in syringe units.

Print, laminate and post at the point of care

7 Medical Director Authorization

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WEIGHT LOSS

Patient Screening & Candidacy

Who qualifies, who does not, and why the documented decline matters as much as the approval

DOCUMENT WL-003	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and intake staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This is the gatekeeping protocol. It defines who is a candidate for pharmacologic weight management in this practice, who is excluded, who must be referred, and how each decision is recorded. A program judged after the fact is judged largely on who it declined.

DOCUMENT THE NO

Approvals are easy to evidence — there is a prescription. Declines usually leave no trace, which means a program that screens carefully looks identical on paper to one that screens nobody. Record every decline, the reason, and the referral made.

2 Candidacy Criteria

Criterion	Standard
Body mass index	BMI thresholds for weight-management pharmacotherapy, with a lower threshold where a weight-related comorbidity is present. Your medical director sets the practice thresholds in line with current labeling and guidance.
Measured, not reported	Height and weight measured in the practice at baseline and at each visit. Self-reported figures are not acceptable for a candidacy decision.
Comorbidity assessment	Hypertension, dyslipidemia, type 2 diabetes, obstructive sleep apnea, and cardiovascular disease identified and recorded.
Prior attempts	History of previous weight-management efforts, including prior pharmacotherapy and bariatric surgery.
Realistic goal	A goal agreed and recorded in the patient's own words, with the expected trajectory and its limits explained.
Capacity to engage	Willingness and ability to attend follow-up, complete monitoring, and self-administer or attend for administration.

3 Exclusions & Referrals

Absolute exclusions

- Personal or family history of medullary thyroid carcinoma, or MEN2.

- Known hypersensitivity to the agent or its excipients.
- Pregnancy, or actively trying to conceive.
- Breastfeeding.
- BMI below the practice threshold with no qualifying comorbidity — including patients who are already at a healthy weight and requesting therapy for cosmetic reasons.
- Age below the threshold set by the medical director for this practice.

Requires evaluation or co-management before proceeding

Finding	Action
History of pancreatitis	Prescriber assessment required. Many practices exclude entirely; if proceeding, document the reasoning.
Gastroparesis or significant GI motility disorder	Generally excluded. Delayed gastric emptying compounds the underlying condition.
Insulin or sulfonylurea therapy	Coordinate with the prescribing physician before initiation. Hypoglycemia risk is real in this group.
Type 1 diabetes	Not a candidate for this program. Refer to endocrinology.
Diabetic retinopathy	Ophthalmology input before initiation; rapid glycemic improvement can worsen retinopathy.
Prior bariatric surgery	Prescriber assessment and coordination with the bariatric team.
Gallbladder disease	Assess and counsel; rapid weight loss increases risk.
Significant renal or hepatic impairment	Prescriber assessment and coordination with the treating physician.
Unstable psychiatric illness	Stabilize and coordinate with the treating clinician before initiating.

4 Disordered Eating Screening

THIS IS THE SCREEN MOST PROGRAMS SKIP

Appetite-suppressing medication given to someone with an active eating disorder is actively harmful. It is also the risk least likely to be documented and most difficult to defend afterwards. Screen every patient, every time, and record it.

Use a validated screening instrument selected and approved by your medical director. Administer at intake and repeat at a defined interval during therapy.

Findings requiring referral before any prescription

- Current or past diagnosis of anorexia nervosa, bulimia nervosa or binge eating disorder.
- Self-induced vomiting, laxative, diuretic or diet pill misuse, current or historical.
- Compulsive exercise, or exercise driven by compensation for eating.
- Rigid food rules, extended fasting, or significant distress around eating.
- Body image concern disproportionate to measured weight.

- A goal weight below a healthy range, or a goal the patient cannot articulate a reason for.
- Family or partner expressing concern about the patient's eating.
- ☐ Screening instrument administered and result recorded at intake.
- ☐ Positive or equivocal screen triggers referral before any prescription is written.
- ☐ Referral pathway to a qualified eating disorder clinician identified and current.
- ☐ Re-screen at the interval set by the medical director during ongoing therapy.
- ☐ New concerns arising during treatment trigger pause and re-referral, not a dose change.

5 Decline & Referral Pathway

- 1 **Explain the reason plainly** and without judgement. Patients decline poorly when they feel dismissed rather than assessed.
- 2 **Offer an alternative** where one exists — referral, co-management with their physician, or reassessment after a specific step.
- 3 **Provide a written referral** where the reason is clinical, addressed to the appropriate specialty.
- 4 **Record the decline:** the criterion not met, the discussion held, the referral made, and the patient's response.
- 5 **Flag the chart** so the same patient is not approved by a different provider next month without the prior assessment being seen.
- 6 **Do not sell an alternative service** as a consolation in the same conversation. It undermines the clinical basis of the decline and reads badly in review.

COMMERCIAL PRESSURE

Weight-loss programs generate recurring revenue and staff may be measured on conversion. State explicitly, in writing and in onboarding, that declining an unsuitable patient is the expected outcome and is never held against anyone. Where that is not culturally true, the screening protocol will not survive contact with a busy Friday.

6 Quick Reference

QUICK REFERENCE CARD

Before the first prescription

1. **Measure** height, weight, BMI, BP, heart rate. No self-report.

2. **MTC / MEN2 question asked and answered.** Recorded verbatim.

3. **Pregnancy, contraception and intent** discussed and documented.

4. **Disordered eating screen** completed. Positive → refer before prescribing.

5. **Insulin or sulfonylurea?** Coordinate with their prescriber first.

6. **Pancreatitis, gastroparesis, retinopathy, bariatric history** reviewed.

7. **Goal agreed in the patient's words** and recorded.

8. **Declining? Document the reason and the referral.**

Print, laminate and post at the point of care

7 Medical Director Authorization

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WL-003 Patient Screening & Candidacy — MedSpa Standards v1.0 · January 2026

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WEIGHT LOSS

Titration & Dose Escalation Management

One set of advance, hold and reduce criteria governing every agent the practice prescribes

DOCUMENT WL-004	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Agent-specific ladders live in WL-001 and WL-002. This protocol governs the decision made at every step of those ladders: advance, hold, or reduce. Applying one written rule across all prescribers is what makes the program consistent and the chart defensible.

ESCALATION IS A CLINICAL DECISION

It is not a scheduled event, a subscription tier, or a response to disappointment. If the reason recorded for an escalation is that four weeks elapsed, the decision has not actually been made.

2 Decision Criteria

ASSESS ALL FOUR BEFORE DECIDING

Domain	Question
Interval	Has the minimum duration at the current dose been completed?
Tolerance	Are GI and other adverse effects absent or mild and stable (WL-005)?
Response	Is progress inadequate relative to the expected trajectory for this dose and duration?
Adherence and intake	Are doses being taken as prescribed, and is oral intake, protein and hydration adequate (WL-007)?

Advance

- Minimum interval completed at the current dose.
- Adverse effects absent, or mild and not worsening.
- Response inadequate against the agreed goal and the expected trajectory.
- Adherence confirmed and nutritional intake adequate.
- Current dose is below the labeled maximum.
- No new contraindication, medication or condition since the last review.

Hold at current dose

- Response is adequate and the patient is progressing toward the goal.

- Adverse effects are present at moderate grade or are not yet stable.
- Interval not completed.
- Adherence uncertain, or intake and protein inadequate — fix these before adding dose.
- Recent intercurrent illness, dehydration, or a planned procedure requiring sedation.
- Patient preference to hold, once the implications have been discussed and recorded.

Reduce or pause

- Moderate adverse effects that are persistent, worsening, or affecting daily function.
- Any severe adverse effect (WL-005), pending evaluation.
- Weight loss occurring alongside poor oral intake, dehydration, or apparent lean mass loss.
- Signs of an emerging disordered eating pattern — pause and refer per WL-003.
- Intercurrent illness with reduced intake, or hospital admission.
- Patient unable to attend monitoring — therapy is paused rather than continued unobserved.

3 Managing Requests to Escalate Faster

This is the most common friction point in a weight-loss program and the most common source of protocol drift. Handle it the same way every time.

- 1 Acknowledge the goal** without agreeing to the method. The patient's frustration is usually real and reasonable.
- 2 Explain the reason for the interval** — tolerance, adverse effect risk, and the fact that faster escalation does not reliably produce faster loss.
- 3 Review the actual data together:** trajectory to date, adherence, intake and protein. The limiting factor is frequently not the dose.
- 4 Address the modifiable factors** per WL-007 and set a defined reassessment point.
- 5 Decline clearly** where criteria are not met. An unclear answer will be read as a maybe and re-asked next week.
- 6 Document the request, the discussion and the decision.** A declined escalation is one of the strongest entries a weight-loss chart can contain.

NEVER ESCALATE OUTSIDE THE LABELED MAXIMUM

Requests to exceed the maximum labeled dose, to stack agents, or to source additional product independently are declined without exception. Where a patient indicates they intend to obtain product elsewhere, document the counseling given and the risks explained, and notify the medical director.

4 Documentation

Each escalation decision — including a decision not to escalate — is recorded with the four domains assessed. This takes two lines and it is the difference between a titration that was reasoned and one that merely happened.

- Current dose, weeks at that dose, and the date of the last change.

- Weight and percentage change since the last review and since baseline.
- Adverse effect grading at the time of the decision.
- Adherence and intake assessment.
- Decision taken and the criteria supporting it.
- Any patient request that differed from the decision, and the discussion held.
- Next review date.

5 Quick Reference

QUICK REFERENCE CARD

Advance · Hold · Reduce

1. **Check all four:** interval, tolerance, response, adherence and intake.

2. **Advance** only if interval done, tolerance good, response inadequate, below max.

3. **Hold** if responding well, symptoms unsettled, or intake inadequate.

4. **Reduce** for persistent moderate or any severe adverse effect.

5. **Pause** for illness, poor intake, missed monitoring, or eating-disorder signals.

6. **Never exceed the labeled maximum.** Never stack agents.

7. **Record the decision either way** — a documented 'no' is worth more than a 'yes'.

Print, laminate and post at the point of care

6 Medical Director Authorization

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WL-004 Titration & Dose Escalation Management — MedSpa Standards v1.0 · January 2026

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WEIGHT LOSS

GI Side Effect Management

Grading, the stepwise management ladder, and the red flags that mean stop today

DOCUMENT WL-005	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Gastrointestinal effects are the main reason patients stop GLP-1 therapy, and most discontinuations are avoidable with structured management. This protocol grades severity, sets a stepwise response, and defines the findings that require the drug to be held and the patient evaluated the same day.

ASK, DO NOT WAIT

Patients frequently under-report nausea because they associate it with the treatment working, or fear the dose being reduced. Ask specifically at every contact and grade the answer.

2 Severity Grading

GRADE AT EVERY CONTACT

Grade	Presentation	Action
Mild	Occasional nausea, tolerating oral intake, normal activity, no dehydration.	Dietary counseling. Continue the current dose. Reassess at the scheduled visit.
Moderate	Persistent nausea, occasional vomiting, reduced oral intake, early dehydration signs, interfering with daily activity.	Hold escalation. Consider dose reduction. Antiemetic per standing order. Reassess within 7 days.
Severe	Intractable vomiting, unable to keep fluids down, orthostatic symptoms, severe abdominal pain, or any red flag below.	Stop the drug. Same-day medical evaluation. Do not manage by telephone.

3 Management Ladder

Step 1 — dietary and behavioural

- Smaller, more frequent meals; stop eating at the first sense of fullness.
- Reduce fatty, fried and highly spiced foods, which empty slowly and worsen symptoms.
- Separate fluids from meals; sip steadily through the day rather than with food.
- Eat slowly and remain upright for a period after eating.
- Prioritize protein even when appetite is low — see WL-007.
- Avoid alcohol, which worsens nausea and dehydration.

Step 2 — symptom-specific measures

Symptom	Approach
Nausea	Dietary measures first. Antiemetic per the medical director's standing order if inadequate. Reassess within a week.
Constipation	Fluid and fiber first, then a stool softener or osmotic laxative per standing order. Persistent constipation with distension and vomiting is a red flag, not a laxative problem.
Diarrhea	Assess hydration. Review other medications and dietary triggers. Persistent diarrhea warrants evaluation for another cause.
Reflux	Meal timing, upright posture, portion size. Acid suppression per standing order if persistent.
Early satiety with inadequate intake	This is a nutritional problem, not just a comfort problem. Address per WL-007 and consider dose reduction.

Step 3 — dose modification

- Hold escalation for any moderate symptom that is not settling.
- Reduce to the previously tolerated dose for persistent moderate symptoms, and remain there for at least the standard interval before reconsidering.
- Extend the interval at the current dose rather than reducing, where the prescriber judges that appropriate.
- Discontinue for severe symptoms, pending evaluation.
- Record the modification and the reasoning in every case.

4 Red Flags

HOLD THE DRUG AND ARRANGE SAME-DAY EVALUATION

Each of these represents a potentially serious complication rather than a tolerance issue.

Finding	Concern
Severe persistent abdominal pain, often radiating to the back, with vomiting	Acute pancreatitis.
Right upper quadrant pain, fever, jaundice, pain after fatty meals	Gallbladder disease — risk is increased during rapid weight loss.
Abdominal distension, absent bowel movements, vomiting, no flatus	Ileus or bowel obstruction.
Inability to keep any fluid down for an extended period, dizziness on standing, reduced urine output	Significant dehydration and acute kidney injury risk.
Vomiting blood, or black tarry stools	Gastrointestinal bleeding.
Shakiness, sweating, confusion — particularly on insulin or a sulfonylurea	Hypoglycemia. Check glucose; follow the emergency protocol.

DEHYDRATION IS THE QUIET ONE

Reduced intake plus vomiting or diarrhea, in a patient already eating very little, causes dehydration and acute kidney injury more often than the dramatic complications do. Assess hydration at every contact where symptoms are present, and have a low threshold for pausing therapy.

5 Documentation & Follow-Up

- ☐ Symptoms asked about specifically at every contact, and the grade recorded.
- ☐ Hydration and oral intake assessed whenever symptoms are present.
- ☐ Every intervention recorded, including dietary counseling given.
- ☐ Dose modifications recorded with reasoning.
- ☐ Red flag screen documented as negative where relevant — not merely omitted.
- ☐ Follow-up interval shortened whenever symptoms are moderate.
- ☐ Any adverse event reported to the manufacturer and to FDA MedWatch where applicable.
- ☐ Patients given written red-flag warning signs and after-hours contact instructions.

6 Quick Reference

QUICK REFERENCE CARD

Patient reports GI symptoms

- Grade it:** mild, moderate or severe.
- Screen the red flags** before anything else.
- Mild:** dietary measures, continue dose, reassess as scheduled.
- Moderate:** hold escalation, consider reduction, reassess within 7 days.
- Severe:** stop the drug, same-day evaluation. Not a phone call.
- Back pain + vomiting → pancreatitis until excluded.**
- Distension + no bowel movement + vomiting → obstruction, not constipation.**
- Assess hydration every time.** It is the most common real harm.

Print, laminate and post at the point of care

7 Medical Director Authorization

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WEIGHT LOSS

Compounded GLP-1 Sourcing & Documentation

Pharmacy vetting, disclosure, lot tracking and the paper trail regulators ask for first

DOCUMENT WL-006	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Medical director, practice manager, prescribing clinicians	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol governs the sourcing, storage, disclosure and traceability of compounded GLP-1 preparations. It is the most scrutinized area of weight-loss practice and the document most likely to be requested by a board, a carrier or an attorney.

LEGAL REVIEW REQUIRED

Whether compounded GLP-1 preparations may lawfully be prescribed or dispensed at any given time depends on federal drug shortage status, FDA enforcement posture, state pharmacy law, and ongoing litigation — all of which change faster than any template. **This protocol does not tell you whether to use compounded product.** It tells you what to document if your medical director, advised by a healthcare attorney, decides that you may.

RECORD THE BASIS, AND REVIEW IT

Whatever the current status, record the specific basis on which compounded product is being used, the date that determination was made, who made it, and the date it will be reviewed. An undated assumption from two years ago is not a basis.

2 Pharmacy Vetting

Category	What it means	Practice implication
503A compounding pharmacy	Compounds for an identified individual patient against a valid prescription.	Patient-specific prescriptions required. Office stock is generally not permitted.
503B outsourcing facility	Registered with FDA, subject to CGMP requirements and FDA inspection. May produce batches without patient-specific prescriptions.	Permits office stock where state law allows. Higher manufacturing standard.

Vetting file — obtain before the first order and refresh annually

- ☐ Current pharmacy or outsourcing facility license for its home state, verified at primary source.
- ☐ Non-resident pharmacy license or registration for your state, where required.
- ☐ FDA registration confirmed for any 503B outsourcing facility.
- ☐ Most recent inspection outcome, and any FDA warning letters or state disciplinary action — checked, not assumed.

- ☐ Sterility and endotoxin testing practices for sterile injectables, in writing.
- ☐ Potency and identity testing, with certificates of analysis available on request.
- ☐ Active pharmaceutical ingredient source and country of origin.
- ☐ Beyond-use dating methodology and its basis.
- ☐ Recall procedure and notification pathway to your practice.
- ☐ Insurance and indemnity position.
- ☐ Named contact for quality queries and adverse event reporting.

REFUSE OUTRIGHT

Product marketed as 'research chemical' or 'not for human use'. Product sourced from an unlicensed reseller, a peptide website, or a social media supplier. Product where the pharmacy will not supply licensure or testing documentation. Semaglutide salt forms such as semaglutide sodium or semaglutide acetate, which are not the same active ingredient as the approved base and have been the subject of specific FDA concern. No commercial pressure justifies any of these.

3 Receiving, Storage & Traceability

- 1 **Inspect on receipt.** Packaging intact, cold chain indicators within range, labeling complete and legible, beyond-use date acceptable for expected use.
- 2 **Reject and quarantine** anything with a compromised seal, a temperature excursion, missing labeling, or visible particulates or discoloration. Do not use pending a decision by the medical director.
- 3 **Log receipt** in the product register: pharmacy, product, concentration, lot, quantity, beyond-use date, receipt date, and receiving staff member.
- 4 **Store per labeling.** Refrigerated, protected from light, in a monitored unit with a daily temperature log.
- 5 **Record every dispense or administration** against the lot: patient, date, dose, and administering or dispensing clinician.
- 6 **Reconcile the register monthly** — quantity received against quantity used, wasted and remaining. Investigate any discrepancy.
- 7 **Log waste and expiry disposal** with a witness signature.

TRACEABILITY IS THE WHOLE POINT

If a compounding pharmacy issues a recall, you must be able to identify every patient who received product from the affected lot, and contact them, within a day. If your records cannot do that, the sourcing program is not adequate no matter how good the pharmacy is.

4 Patient Disclosure & Consent

Patients frequently do not know they are receiving a compounded preparation rather than branded product, and many assume the two are equivalent. Disclosure must be explicit, written, and made before the first dose.

Required disclosure elements

- That the preparation is compounded and is not an FDA-approved product.
- That compounded preparations are not reviewed by FDA for safety, efficacy or manufacturing quality in the way approved products are.
- The name of the compounding pharmacy or outsourcing facility.
- The concentration supplied, and that it may differ from branded presentations.
- That the branded FDA-approved alternative exists, and its comparative cost.
- Known risks, including that potency and content may vary between compounders.
- Reported dosing errors arising from unit-versus-milligram confusion, and the specific instructions the patient must follow.
- How to report a suspected problem, and to whom.

DOSING ERROR PREVENTION

Overdoses have been reported where patients or staff confused milligrams with insulin syringe unit markings, or where a new vial had a different concentration from the last. Every dispensed vial must be labeled with the concentration and the dose in both milligrams and the corresponding volume. Confirm the concentration every time a new lot is opened. Never let a patient calculate their own conversion.

5 Adverse Events & Recall Response

- 1 **Quarantine immediately.** Remove the affected lot from service and physically separate it.
- 2 **Notify the medical director** the same day.
- 3 **Identify every patient** who received product from the affected lot using the product register.
- 4 **Contact affected patients** with clear instructions on whether to stop, and what symptoms to watch for.
- 5 **Notify the compounding pharmacy** and request their investigation and response in writing.
- 6 **Report to FDA MedWatch** where an adverse event or quality defect is involved.
- 7 **Retain the product** and its packaging rather than discarding it — it may be needed for investigation.
- 8 **Document everything**, including the decision-making and the timeline, and review whether the pharmacy remains suitable.

6 Records to Retain

- ☐ Pharmacy vetting file, current and superseded versions, with annual refresh dates.
- ☐ Documented basis for compounded use, with its determination date and review date.
- ☐ Certificates of analysis where obtained.
- ☐ Product register: receipt, lot, beyond-use date, dispensing and reconciliation.
- ☐ Refrigerator temperature logs.

- ☐ Signed patient disclosure and consent for every patient receiving compounded product.
- ☐ Waste and expiry disposal logs with witness signatures.
- ☐ Adverse event and recall records with the full response timeline.

7 Quick Reference

QUICK REFERENCE CARD

Compounded product — non-negotiables

1. **Vetting file complete** before the first order; refreshed annually.
2. **Basis for compounded use recorded and dated.** Review on schedule.
3. **Refuse** research chemicals, unlicensed resellers, salt forms.
4. **Log every lot in and every dose out.** Recall readiness in one day.
5. **Written disclosure signed** before the first dose. Every patient.
6. **Label mg and volume.** Never let anyone convert from syringe units.
7. **Confirm concentration** whenever a new lot is opened.
8. **Recall: quarantine, identify, contact, report, retain the product.**

Print, laminate and post at the point of care

8 Medical Director Authorization

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WEIGHT LOSS

Nutrition, Muscle Preservation & Lifestyle

Protein, resistance training, and the referral boundary of a practice that is not a dietetics clinic

DOCUMENT WL-007	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Rapid weight loss without adequate protein and resistance training costs lean mass, and lean mass is difficult to regain. This protocol sets the counseling standard, defines what the practice does and does not provide, and identifies when a patient must be referred to a registered dietitian.

KNOW THE BOUNDARY

General nutrition counseling supporting a medical treatment is within reach of most practices. Individualized medical nutrition therapy — particularly for patients with diabetes, renal disease, or a history of disordered eating — is a registered dietitian's scope in many states. Your medical director must define the boundary for your practice and state, and staff must know where it sits.

2 Protein & Intake

Appetite suppression is the mechanism of the drug, which means inadequate protein intake is the default outcome unless it is actively managed. Set a target at initiation rather than responding to lean mass loss later.

Element	Counseling standard
Protein target	An individualized daily target set by the prescriber, expressed in grams and distributed across meals. Your medical director sets the practice framework; body weight and comorbidity determine the individual figure.
Protein first	Eat protein at the start of each meal, before the sense of fullness arrives. This single instruction does more than any other.
Distribution	Spread across the day rather than concentrated in one meal; small, frequent, protein-led eating suits reduced appetite better.
Hydration	A daily fluid target, sipped through the day and away from meals. Dehydration is common and drives much of the fatigue reported.
Fiber	Adequate fiber alongside fluid to manage constipation before it needs medication.
Micronutrients	Assess adequacy where intake is very low. Supplementation decisions sit with the prescriber.
Alcohol	Reduce or avoid. It worsens nausea, dehydration and hypoglycemia risk, and displaces protein calories.

VERY LOW INTAKE IS A CLINICAL FINDING

A patient eating a few hundred calories a day and losing weight rapidly is not a program success. Assess for dehydration, protein inadequacy and lean mass loss, address per this protocol, and consider dose reduction per WL-004. Never congratulate a patient on an intake level that is harming them.

3 Resistance Training

- Resistance training is recommended to every patient at initiation, not introduced after lean mass loss appears.
- Recommend a realistic starting frequency covering major muscle groups, appropriate to the patient's current activity and any physical limitation.
- Progressive overload matters more than modality — body weight, bands, machines or free weights are all acceptable starting points.
- Combine with general activity guidance; steps and daily movement support adherence but do not substitute for resistance work.
- Refer to a qualified exercise professional where the patient has cardiac, orthopedic or mobility considerations.
- Record the recommendation given and the patient's response at each review.

FRAMING MATTERS

Patients hear resistance training as an aesthetic suggestion. Frame it as part of the treatment: the medication reduces intake, and training is what determines whether the weight lost comes from fat or from muscle. Patients act on that framing far more often.

4 Tracking

Measure	Interval	Why
Weight	Every visit	Trajectory against the agreed goal.
Body composition where available	At baseline and at set intervals	Distinguishes fat loss from lean mass loss. Scale weight alone cannot.
Circumference measurements	At baseline and at set intervals	Simple, reproducible, and useful when composition analysis is unavailable.
Protein intake	Every visit	The variable most likely to be inadequate.
Resistance training frequency	Every visit	Adherence to the recommendation given.
Functional markers	Periodically	Strength, stair climbing, grip. Falling function during weight loss is a warning sign.

SCALE WEIGHT IS A POOR SOLE ENDPOINT

A program that reports only scale weight cannot distinguish an excellent outcome from a harmful one. Where body composition analysis is unavailable, use circumference and functional markers alongside weight rather than weight alone.

5 Referral & Scope Boundaries

Refer to a registered dietitian

- Any positive or equivocal disordered eating screen — before continuing therapy, per WL-003.
 - Persistent inability to meet protein targets despite counseling.
 - Diabetes requiring coordinated medical nutrition therapy.
 - Renal impairment, hepatic disease, or another condition with specific dietary requirements.
 - Prior bariatric surgery.
 - Pregnancy occurring during the program — therapy is stopped and the patient referred.
 - Evidence of lean mass loss or falling functional capacity.
 - Any patient requesting a detailed individualized meal plan, where that exceeds your practice's defined scope.
- ☐ A named registered dietitian or referral pathway identified and current.
 - ☐ Referral criteria known to all clinical staff.
 - ☐ Referrals made in writing and recorded, with outcomes followed up.
 - ☐ Staff clear on what nutrition guidance they may and may not provide.
 - ☐ Written patient education materials reviewed and approved by the medical director.

6 Quick Reference

QUICK REFERENCE CARD

Every weight-loss visit

- Ask the protein number.** Not 'are you eating well?'
- Protein first at every meal** — the highest-yield instruction you can give.
- Check hydration and fiber.** Prevents most constipation.
- Resistance training** — frame it as part of the treatment, not a suggestion.
- Track more than the scale.** Composition, circumference, function.
- Very low intake is a finding**, not a success. Act on it.
- Refer** for eating disorder signals, diabetes, renal disease, or lean mass loss.

Print, laminate and post at the point of care

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WEIGHT LOSS

Lipotropic & B12 Injection Protocol

MIC, B-complex and lipotropic blends: technique, frequency limits and honest claims

DOCUMENT WL-008	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Clinical staff administering injections	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol covers lipotropic blends, vitamin B12 and B-complex injections offered alongside or independently of pharmacologic weight management. The clinical risk is modest; the regulatory risk sits almost entirely in how these services are described and advertised.

THE RISK HERE IS THE CLAIM

Evidence that lipotropic injections produce meaningful weight loss independent of diet is limited. Advertising them as a fat-burning or weight-loss treatment is where practices get into difficulty — with advertising regulators and, in several states, with licensing boards. Sell what you can substantiate.

2 Formulations & Indications

Component	Typical rationale	Documentation standard
Cyanocobalamin or methylcobalamin (B12)	Treatment of documented deficiency; supplementation where absorption is impaired.	Deficiency-based use should be supported by a lab result. Non-deficiency use is elective and must be described as such.
Methionine, inositol, choline (MIC)	Marketed as supporting fat metabolism.	Evidence for independent weight-loss effect is limited. Counsel and advertise accordingly.
B-complex	General supplementation.	Elective. No weight-loss claim should be attached.
L-carnitine	Marketed as supporting fat utilization.	Evidence limited. Same counseling standard.

- The formulation used, its components and concentrations, must be recorded and approved by the medical director.
- Compounded blends are subject to the sourcing and documentation requirements in WL-006.
- Any change of formulation or supplier requires re-approval, not a substitution at the shelf.

3 Screening & Administration

- ☐ Allergy screen including sensitivity to any component and to preservatives.
- ☐ Pregnancy and breastfeeding status.

- ☐ Renal or hepatic impairment, which may affect suitability.
- ☐ Current medications and supplements, to avoid duplication and interaction.
- ☐ For B12: relevant lab result where use is deficiency-based.
- ☐ Consent obtained, including an explicit statement of what the injection is and is not expected to achieve.

- 1 **Verify the product**, concentration, lot and expiry or beyond-use date against the order.
- 2 **Inspect the vial** for particulates, discoloration or damage. Discard if in doubt.
- 3 **Select the site** — deltoid, ventrogluteal or vastus lateralis — appropriate to the volume being given.
- 4 **Prepare aseptically** and administer by intramuscular injection using appropriate needle length for the patient's build.
- 5 **Rotate sites** between doses and record the site used each time.
- 6 **Observe** for the practice's standard post-injection interval; a first dose warrants a longer observation.
- 7 **Record** product, lot, dose, site, administering clinician and patient response.

FREQUENCY LIMITS

Set a maximum frequency per formulation, approved by the medical director, and apply it consistently. Weekly is common for elective supplementation. Escalating frequency because a patient wants faster results has no clinical basis and undermines the position that these are medically supervised services.

4 Counseling & Marketing Standards

What to say

- Describe the components and their generally understood role plainly.
- State clearly where evidence for a weight-loss effect is limited.
- Position these injections as adjunctive to nutrition and activity, never as a substitute for either.
- Where used for documented B12 deficiency, explain that specifically — this is the one clearly evidence-supported indication in the group.
- Set expectations before the first injection, not after a course has been purchased.

What not to say or publish

- Claims that the injection burns fat, melts fat, or causes weight loss on its own.
- Specific pounds-lost figures attributed to the injection.
- Metabolism-boosting claims presented as established fact.
- Energy-level claims presented as guaranteed outcomes.
- Before-and-after imagery attributing results to the injection where the patient was also on pharmacotherapy or a structured diet.
- Package deals framed around a promised amount of weight loss.

CROSS-ATTRIBUTION

Showing a before-and-after from a patient on semaglutide while marketing lipotropic injections attributes one treatment's result to another. It is deceptive even if both images are real and unretouched. Attribute results to the treatment that produced them.

5 Documentation

- Formulation, components and concentration, with medical director approval on file.
- Indication for this patient, and any supporting lab result.
- Consent including the expectation-setting discussion.
- Every administration: date, product, lot, expiry, dose, site, clinician.
- Frequency tracked against the approved maximum.
- Adverse reactions and actions taken.
- Marketing materials reviewed against the claim standards above and re-reviewed annually.

6 Quick Reference

QUICK REFERENCE CARD

Lipotropic & B12 injections

1. **Verify product, lot, concentration, expiry** against the order.

2. **Allergy, pregnancy, renal and medication screen** every time.

3. **Aseptic IM technique**, rotate sites, record the site.

4. **Observe** after injection — longer for a first dose.

5. **Respect the approved frequency limit**. No escalation on request.

6. **Say what is evidence-supported**. B12 deficiency is; fat-burning is not.

7. **Never attribute another treatment's results** to these injections.

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WEIGHT LOSS

Monitoring, Labs & Follow-Up Schedule

The cadence, the panels, and what happens when a patient stops attending

DOCUMENT WL-009	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Follow-up is what makes this a medical program rather than a supply arrangement. This protocol sets the visit cadence, the laboratory panels and their intervals, the escalation pathway for abnormal results, and the response when a patient with an active prescription disengages.

PRESCRIPTIONS DO NOT OUTLIVE MONITORING

A patient who cannot or will not attend monitoring does not continue on therapy. The prescription is contingent on the follow-up, and staff should be able to say so without needing to escalate the conversation.

2 Visit Cadence

Phase	Interval	Required at each visit
Baseline	Before first dose	Full evaluation per WL-003, measured vitals and anthropometrics, baseline labs, consent, education.
Titration phase	At or before each dose escalation	Weight, BP, resting heart rate, GI symptom grading, adherence, intake and protein, escalation decision recorded per WL-004.
Early maintenance	At the interval set by the medical director	As above, plus body composition or circumference, and interim labs where indicated.
Established maintenance	At a defined longer interval, not exceeding the maximum set by the medical director	As above, plus full review of goal, therapy duration and exit planning per WL-010.
Annual	Every 12 months on therapy	Comprehensive review: full labs, contraindication re-screen, disordered eating re-screen, consent refresh, and documented decision to continue.

TELEHEALTH VISITS

Where follow-up occurs by telehealth, the practice must still obtain objective measurements — a documented home weight, and blood pressure where the patient has a cuff — and must define which assessments require an in-person visit. Record the modality of every encounter. State telehealth rules for prescribing vary and must be confirmed.

3 Laboratory Monitoring

The panel below is a framework. Your medical director sets the practice panel, the intervals, and the thresholds, taking into account each patient's comorbidities.

FRAMEWORK — MEDICAL DIRECTOR TO FINALIZE

Test	Baseline	Repeat	Rationale
Comprehensive metabolic panel	Yes	At set intervals	Renal and hepatic function; electrolytes, particularly where GI losses occur.
HbA1c	Yes	At set intervals	Glycemic status; identifies undiagnosed diabetes and tracks improvement.
Fasting glucose	Yes	As indicated	Baseline glycemic assessment.
Lipid panel	Yes	At set intervals	Cardiovascular risk tracking.
Complete blood count	Yes	As indicated	General baseline.
TSH	Yes	As indicated	Thyroid dysfunction as a contributor to weight change.
Lipase	As indicated	If symptoms arise	Not routinely required; obtained where pancreatitis is suspected.
Pregnancy test	Where applicable	Per medical director	Before initiation in patients who could become pregnant.
Vitamin and micronutrient studies	As indicated	As indicated	Where intake is very low or deficiency is suspected.

4 Abnormal Results & Escalation

- 1 All results reviewed by a clinician** within a defined timeframe of receipt. Results are not filed unreviewed, and 'no news' is not a system.
- 2 Classify** the result: within expected range, abnormal requiring monitoring, abnormal requiring action, or critical.
- 3 Critical results are actioned the same day**, with the patient contacted directly rather than by message.
- 4 Notify the patient** of every result, normal or otherwise, within the practice's stated timeframe.
- 5 Communicate with the primary care physician** for any finding outside the scope of this program — with the patient's consent, and in writing.
- 6 Hold or stop therapy** where the result warrants it, per WL-004.
- 7 Document** the review, the classification, the action, the notification and the onward referral.

THE UNREVIEWED RESULT

An abnormal result sitting unread in an inbox is one of the most common and most indefensible failures in outpatient medicine. Assign a named owner for result review, with named cover for absence, and audit the queue on a schedule.

5 Non-Attendance & Lost to Follow-Up

A patient on an active prescription who stops attending is a clinical risk and an accountability problem. Define the response rather than letting the chart go quiet.

- ☐ Missed appointments logged, with a contact attempt made and recorded.
- ☐ A defined number of contact attempts through more than one channel before a patient is considered disengaged.
- ☐ No further prescription issued or product dispensed once the follow-up interval has lapsed beyond the defined limit.
- ☐ Written notice to the patient explaining that therapy cannot continue without monitoring, and what they must do to resume.
- ☐ Primary care physician notified where clinically appropriate and consented.
- ☐ Formal discharge from the program after a defined period, with a discharge letter and summary per WL-010.
- ☐ Chart flagged so a lapsed patient is re-evaluated rather than simply re-supplied.

6 Quick Reference

QUICK REFERENCE CARD**Monitoring essentials**

1. **Baseline labs before the first dose.** No exceptions.
2. **Weight, BP and resting heart rate** at every visit — measured.
3. **GI grading, adherence, protein** every visit.
4. **Named owner for result review**, with cover, and an audited queue.
5. **Critical result → same-day direct contact.**
6. **Findings outside program scope → write to primary care.**
7. **No monitoring, no prescription.** Say it plainly and early.
8. **Annual comprehensive review** with a documented decision to continue.

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WEIGHT LOSS

Discontinuation, Maintenance & Weight Regain

Planning the exit at intake, and handling the conversation nobody prepares for

DOCUMENT WL-010	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Prescribing clinicians and clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Every patient stops eventually — by choice, by cost, by supply, by side effect, or by reaching their goal. This protocol covers maintenance strategy, planned discontinuation, the regain conversation, and the exit documentation that protects both the patient and the practice.

THE REGAIN CONVERSATION BELONGS AT INTAKE

Weight regain after stopping GLP-1 therapy is common and substantial, and patients are frequently told this for the first time when they are already off the drug and regaining. That sequence produces complaints, refund demands and reviews. Have the conversation before the first prescription and document it.

2 Maintenance Strategy

Approach	Description	Considerations
Continue at the effective dose	Ongoing therapy at the dose that achieved the goal.	Simplest and best supported. Requires ongoing monitoring, cost and supply.
Reduce to a lower maintenance dose	Step down to the lowest dose that holds the result.	Reasonable trial. Monitor for regain and be prepared to step back up.
Extend the dosing interval	Same dose, longer interval between administrations.	Off-label in approach. Requires explicit prescriber decision and documentation.
Planned discontinuation	Stop therapy with a structured lifestyle plan in place.	Requires realistic counseling on regain and a defined re-engagement pathway.

Whichever is chosen, record the rationale, the monitoring plan, and the criteria that would prompt a change of approach.

3 Planned Discontinuation

- 1

Establish the reason — goal achieved, cost, side effects, pregnancy planning, supply, or patient preference. The reason shapes the plan.

- 2 **Decide taper versus stop.** There is no withdrawal syndrome, but a stepped reduction gives a clearer view of where appetite settles and makes regain easier to detect early.
- 3 **Reinforce the lifestyle foundation** before the last dose, not after: protein target, resistance training, and a realistic eating pattern per WL-007.
- 4 **Set the monitoring plan.** Follow-up weights at defined intervals after stopping, with a specific point at which regain is reassessed.
- 5 **Agree the re-engagement threshold** in advance — the amount of regain at which the patient returns for review rather than waiting until they are back at baseline.
- 6 **Handle pregnancy planning specifically**, using the discontinuation interval set by your medical director, and coordinate with the patient's obstetric care.
- 7 **Write to the primary care physician** with a summary, with consent.
- 8 **Issue a discharge summary** to the patient covering therapy received, results achieved, and the plan going forward.

4 The Regain Conversation

Deliver this at intake, repeat it at maintenance planning, and repeat it again at discontinuation. Consistency between what was said at the start and what is said at the end is what determines whether regain becomes a complaint.

- Obesity is a chronic, relapsing condition, and these medications treat it while they are being taken.
- Substantial regain after stopping is common and is a physiological response, not a personal failure and not a defect in the treatment.
- Maintaining lean mass through protein and resistance training improves the odds after stopping — which is why WL-007 matters most at the exit.
- Regain is not a reason for shame, and returning for review is expected rather than an admission of failure.
- Re-initiation is possible and follows the full titration ladder again, not a resumption at the previous dose.

DO NOT PROMISE PERMANENCE

Marketing or counseling that implies the result is permanent once achieved is not supportable and is the root of most weight-loss program disputes. Neither staff nor advertising material should suggest it. Review your marketing against this standard specifically.

5 Supply Interruption & Shortage

- ☐ A written contingency plan for supply interruption, known to clinical staff.
- ☐ Patients informed proactively rather than discovering it at a scheduled appointment.
- ☐ Prescriber decision on whether to hold, reduce, or switch agents — documented per patient rather than applied as a blanket instruction.
- ☐ Switching agents follows the cross-titration requirements in WL-002, including restarting the ladder.
- ☐ Patients explicitly warned against sourcing product independently, with the risks explained and the counseling recorded.

- ☐ Alternative sourcing evaluated only through the WL-006 vetting process — never expedited because of urgency.
- ☐ Interruption, duration and restart decisions documented for every affected patient.

RESTART AFTER A GAP

Tolerance is lost during an interruption. A patient resuming after a significant gap generally re-initiates at a lower dose and re-titrates. Resuming at the pre-interruption dose is a common cause of severe GI adverse effects.

6 Exit Documentation

- Reason for discontinuation, and whether it was planned or patient-initiated.
- Final dose, total duration of therapy, and total weight change from baseline.
- Body composition change where measured.
- The regain counseling delivered, and when it was first delivered.
- Maintenance plan agreed, including the re-engagement threshold.
- Post-discontinuation follow-up arranged.
- Discharge summary issued to the patient.
- Letter to the primary care physician, with consent.
- Any outstanding referrals, and confirmation they were made.

7 Quick Reference

QUICK REFERENCE CARD**Stopping therapy**

1. **Regain counseling at intake** — not at the exit.
2. **Establish the reason** for stopping; it shapes the plan.
3. **Reinforce protein and resistance training before the last dose.**
4. **Agree a re-engagement threshold** in advance.
5. **Pregnancy planning** — use the medical director's discontinuation interval.
6. **Supply interruption:** per-patient prescriber decision, documented.
7. **Restarting after a gap:** lower dose, re-titrate. Never resume at the old dose.
8. **Discharge summary + letter to primary care.** Close the loop.

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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